جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : مياده محمد الشناوي علي عطيه
المحافظة : الشرقيه **الحي / المدينة :** الزقازيق **العنوان :** الشرقيه
رقم البطاقة : 28311141300688 **رقم التليفون :** 01280434286 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 14-11-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مياده محمد الشناوي علي عطيه
National ID: 28311141300688

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID : Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : مبادة محمد الشناوى على
Sex: Female
3- Medical number: 83159638
Old Med : 3206-2023
Old Med :

2-ID number: 28311141300688
Patient Age : 40 Y

MDT

Family history	Family History +ve	Phone	Phone 01229881075	BMI	BMI	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status MARRID	NRS Score	NRS Score	GROUP	GROUP C	Date of MDT	Date Of MDT 11/05/2024
Co morbidity	Co Morbidity -ve	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :	Mets Workup :
RT Breast speculated solid soft mass two satellite ill booed cortex axillary LN	rt ovarian cyst

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT:	8/8 ER COMMENT:	7/8 PR COMMENT:	3 Her 2 Neu COMMENT:	SISH COMMENT :	15

T	N	M
4d	2	0

Sonomammography

RT Breast speculated solid soft tissue mass
two satellite & iliac cortex axillary LN

PATIENT HISTORY

MDT 11/9: AC x4 then taxotere -phesgo x5 then MM but pt shifted to taxol weekly due to poor tolerance ended 26/2/2024
o/e: good clinical response , partial regression, localized skin edema
RT MRM:DCIS 3%
no resdiual viable invasive component
free resected margins
free skin
LN 2/12

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR
ADJ TDM-1 X 12
AI/ZOLADEX
PORT &REGIONAL LYMPHATICS (RESIDUAL n2)

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: مياده محمد الشناوي على	
MR. : 4748	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient underwent a bone scan, abdominal ultrasound, and chest X-ray as part of staging work-up; a mammogram performed in November 2025 was reported as negative.

The patient was prescribed Arimidex (anastrozole) with supportive medication for the next two months to be obtained from the pharmacy, and a Zoladex (goserelin) injection is scheduled for 11 August 2026.

The patient is a 42-year-old, premenopausal individual with right-sided breast cancer, clinical stage cT4d N2 M0, invasive ductal carcinoma, estrogen-receptor and progesterone-receptor scores 8/8, HER2 3+. Neoadjuvant chemotherapy consisted of four cycles of doxorubicin/cyclophosphamide (AC X4) followed by four cycles of docetaxel plus trastuzumab (TXT + HP X4) completed on 3 April 2024. The patient subsequently underwent right modified radical mastectomy with pathology showing ypTis ypN1 (2/12 nodes positive). Adjuvant therapy included trastuzumab (Kadcyla) and initiation of an aromatase inhibitor (AI) combined with Zoladex in May 2024. The patient received three cycles of Arimidex plus supportive therapy, with the next Zoladex dose planned for 10 November. A breast-sparing surgery was performed in May 2026 with no residual disease. The patient has completed four cycles of AC, four cycles of TXT + HP, and three cycles of Arimidex plus supportive therapy to date.

تحريرا في ,

التاريخ :	18-09-2026	**الوقت :**	

الطبيب المعالج : مايكل جمال فوزى

يعتمد ,

Patient Name	: الأستاذه / مياده محمد الشناوى على عطيه	Request No.	: 13007691
Patient No.	: 28311141300688	Request Date	: 12/08/2023
Path No.	: 2629/23	Verification Date	: 23/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 39 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Righ breast mass.
Nature of Specimen: Tru-cut biopsy, collected at12/8/2023 .

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Moderate nuclear reaction in 70 %tumor cells (Allred score 7/8).
- Her2: Strong membrane reaction in>10 tumor cells (score +3).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:
Right breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score +3).
- Ki67: Positive in 25% of tumor cells.

Signature

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – ميت عقبة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mist El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / مياده محمد الشناوى على عطيه	Request No.	13007691
Patient No.	28311141300688	Request Date	12/08/2023
Path No.	2629/23	Verification Date	23/08/2023
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	39 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof Dr Manal El mahdy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الأستاذه / ميادة محمد الندوى على عطيه	Request No.	: 13008810
Patient No.	: 28311141300688	Request Date	: 03/04/2024
Path No.	: 1229/24	Verification Date	: 21/04/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:
* Procedure: Right MRM measured 24x18x5 cm, with covering skin ellipse measured 18x10 cm.
* Tumor extent: A greyish white tumor bed site.
* Size: 6x5x4 cm.
* Skin: Nipple & areola looked unremarkable grossly.
* Distance from margins: 1 cm away from the deep margin.
* Lymph nodes number and size: 12 L.Ns, up to 1x1x0.5 cm.
* 11 Block:
1:6- T.B.S. 7- Nipple & areola. 8- Deep margin.
9:11- Lymph nodes.
* Gross was done by; Dr. Heba Hassan.

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Actual tumor cellularity: 0%.
* Ductal carcinoma in situ: Focally detected (3%).
* Skin: Free.
* Lymph nodes: Positive metastatic deposits in two lymph nodes, L.N (2/12), the largest metastasis measuring 1 cm in maximum dimension.
* Resection Margins: Free.
* Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:
Right breast cancer post NACT, Right modified radical mastectomy specimen:

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مياده محمد الشناوى على عطيه	Request No.	: 13008810
Patient No.	: 28311141300688	Request Date	: 03/04/2024
Path No.	: 1229/24	Verification Date	: 21/04/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- Ductal carcinoma in situ (DCIS) (3 %).
- No residual viable invasive component.
- Resection Margins: Free.
- Skin: Free.
- Lymph nodes: Positive metastatic deposits in two lymph nodes, L.N (2/12).
- Histologic response to therapy: Pathological partial response to therapy.
- Stage: ypTis, ypN1a.
- RCB: 1.627, RCB class II.

Signature
Prof. Dr. Aml Hareedy.
Dr. Sagda El-Kady.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ZAGAZIG UNIVERSITY HOSPITALS
RADIOLOGY DEPARTEMENT

Patient name : MAYADA MOHAMMED ELSHENAWY
DATE : 8-8-2023

MAMMOGRAPHY OF BOTH BREASTS :

=Right breast : harbor spiculated dense lesions at the lower outer quadrant with fine calcifications .
= Increase both breasts parenchymal density(ACR :C) with no focal lesions at left side .
= Normal nipple areola complex
= Normal skin thickness.
= Multiple non significant axillary LN shadows .

Complementary US :

= RT breast:An ill defined hypoechoic , indistinct margin spiculated soft tissue lesion with posterior shadowing seen opposite 8 O'clock with internal microcalcifications (measures 13x12mm) with adjacent two small satellites measure (11x6 mm) and (6x5 mm).
= Increase both breasts parenchymal echogenicity mainly at retro areolar region.

=Left breast no solid or cystic masses,
=Normal skin thickening,
=No ductal dilatation.
= Right side few enlarged axillary LNs one of them show eccentric fatty hilum and increase cortical thickness measure 8 mm and LN measure 21 mm .
=No significantly enlarged axillary lymph nodes at left side .

OPINION:

Right breast spiculated solid soft tissue mass with two satellites and thickened cortex axillary LN BIRADs V (malignancy suggestion) for histopathological biposy.

Thanks,
Dr. Basma Kamal.MD
Dr.Rahma Hossam
Dr. Lamiaa Haitham

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: Mayada Mohamed Alshenawy Ali Atteya	Date : 24/08/2023
Exam : C.T CHEST	Age 39 Year(s)
Medical ID: 3206/2023	41270

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. ***Chest:***

- Right breast edema pattern in the form of interstitial edema and increased skin thickness.
- Right axillary level I and II pathologically enlarged lymph nodes the largest is the former measures 2.6 x 2.1 cm.
- Solitary fissural based pulmonary nodule is noted at apical segment of right lower lung lobe measuring 5 mm.
- Small emphysematous bulla is seen at the apical segment of right lower lung lobe measuring 6mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities. Calcified hepatic lesion is seen at sup capsular at segment VI measuring about 1.2x0.9 cm likely old granulomatous infection.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs apart from two functional right ovarian cysts the largest is measuring about 2.7x 2.8 cm.
- No abdominal or pelvic masses or collections.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Mayada Mohamed Alshenawy Ali Atteya	Date : 24/08/2023
Exam : C.T CHEST	Age : 39 Year(s)
Medical ID: 3206/2023	41270

- Fat containing umbilical hernia is noted.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Right breast edema pattern with pathological ipsilateral axillary lymph nodes as described.***
- ***Solitary right lower lobe pulmonary nodule likely benign in nature for follow up.***
- ***No definite hepatic or osseous metastatic deposits.***

Dr.Maie Khattab, MSc.

Signature Prof. Amr Farouk
01/09/2023 02:01 م
Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Mayada Mohammed Elshenawy Ali	Date : 19-11-2025 12:41
Branch : Dokki	Medical No. : 253505
Ref.Physician Prof : Gostaf Rosch Hospital	Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 42-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Mayada Mohamed Alshenawy Ali Atteya	Date : 26/02/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 3206/2023	Pat. ID: 41270

LEFT BREAST MAMMOGRAPHY WITH BILATERAL US

Technique

- LEFT breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions, small operative bed seroma about 36x1mm.
- Free right supra and infra regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

26/02/2025 1:30 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]